

Response to reviewers

Point-by-point response table

Foundations of Clinical Research · Session 8 (8.2) and the peer-review swap after the course · Template version 1.0

Journal: [Journal name]
Manuscript ID: [e.g. JRNL-2026-0123]
Title: [Full title]
Date: [DD/MM/YYYY]
Decision: ☐ Minor revision ☐ Major revision ☐ Reject and resubmit

Tone and method (delete this box)

- Wait a day before replying. Then thank the reviewers once, briefly. They worked for free.
- Answer EVERY comment, in order, numbered (R1.1, R1.2 ...). Copy each comment verbatim.
- Say what you changed and where (page and line in the revised manuscript), and quote the new text.
- Agree where you can. If you disagree, do it politely, with evidence or references, and offer a compromise (e.g. a sentence in the limitations). Never argue about the reviewer.
- Show changes in the manuscript with tracked changes or highlighting, as the journal asks.
- Re-check that the abstract, tables and text still match after the changes.

Letter to the editor

Dear Dr [Editor], thank you for the opportunity to revise our manuscript “[Title]”. We are grateful to the reviewers for their helpful comments. We have addressed every point below and believe the manuscript is clearer as a result. The main changes are: [1–3 main changes]. Page and line numbers refer to the revised manuscript with tracked changes.

Worked examples (fictional)

Two agreements and one polite disagreement, for an imaginary cohort paper on fluid balance.

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R1.1	The authors state that a positive fluid balance increases mortality. This is an observational study and cannot show causation.	Thank you; we agree. We have replaced causal wording throughout with “was associated with”, including in the title and abstract, and added confounding by indication to the limitations.	Title p1; Abstract p2 l.31; Discussion p14 l.312–318
R1.2	How were missing lactate values handled?	Lactate was missing for 23 of 412 patients (5.6%). We now report this in Table 1 and the Methods. The main analysis used multiple imputation; a complete-case analysis gave a similar result (OR 1.21 vs 1.18), shown in new Supplementary Table S3.	Methods p8 l.170–176; Table 1; Suppl. Table S3
R2.1	Please add a subgroup analysis by age group.	We appreciate the suggestion. Age subgroups were not pre-specified, and with 68 deaths the analysis would have very low power and a high risk of a chance finding. We therefore respectfully prefer not to add it, and now mention age as a possible effect modifier for future studies.	Discussion p15 l.340–343

Comments from the editor

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
E.1	[Paste comment]	[Response]	[p __, l. __]
E.2	[Paste comment]	[Response]	[p __, l. __]
E.3	[Paste comment]	[Response]	[p __, l. __]

Reviewer 1

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R1.1	[Paste comment]	[Response]	[p __, l. __]
R1.2	[Paste comment]	[Response]	[p __, l. __]
R1.3	[Paste comment]	[Response]	[p __, l. __]

Reviewer 2

#	Reviewer comment (verbatim)	Our response	Change made (page, line)
R2.1	[Paste comment]	[Response]	[p __, l. __]
R2.2	[Paste comment]	[Response]	[p __, l. __]
R2.3	[Paste comment]	[Response]	[p __, l. __]